



GENEVIEVE APP™

**Safety from roots to every
journey**

GENEVIEVE HEALTH™

MENTAL HEALTH PRACTICE SUPPORT

A connected staff, client, reception and emergency-support program

Prepared for	Irene
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Document status	Confidential preliminary proposal for discussion and controlled development

Purpose: to stop staff, clients, messages, appointments, referrals and urgent safety matters from falling between people, inboxes, shifts or periods of leave.

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GENEVIEVE HEALTH™

Mental Health Practice Support



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Letter to Irene

Dear Irene,

I have prepared this proposal because your recent return from a well-earned month away exposed a serious operational problem: important information about staff availability, clients, appointments and follow-up work is not reaching you early enough. A staff member is now away for two weeks, yet you returned without a reliable handover or a complete picture of what had happened during your absence.

This is not about blaming staff. It is about recognising that capable people can still be let down by disconnected calendars, emails, text messages, verbal handovers, paper notes and tasks that do not have a visible owner. When information is scattered, problems are often discovered only after a client has missed an appointment, a call has gone unanswered, a referral has remained unallocated, a clinician has become unavailable or an emergency has already escalated.

GENEVIEVE HEALTH™ Mental Health Practice Support is proposed as one connected operational safety system for management, clinicians, support staff, reception and clients. Its purpose is to show what is happening now, what changed while someone was away, what is overdue, who owns each action, when it must be escalated and whether the matter was actually resolved.

The system is not intended to replace clinical judgement, professional responsibilities, emergency services or existing approved policies. It is designed to make approved information visible sooner, give every important task a responsible person and create a clear, time-stamped trail from detection through to closure.

The central rule is simple: an alert is not completed merely because it was sent. Every significant matter must move through the stages detected, assigned, accepted, actioned, outcome recorded, reviewed and closed.

I would like to work with you to turn this into a controlled, secure and practical program that reflects how your practice actually operates. The first release would concentrate on staff leave and coverage, return-from-leave handovers, client care gaps, missed appointments, reception callbacks, referrals, weather disruptions, urgent safety escalation and a management dashboard that gives you a complete picture within seconds.

Yours sincerely,

Tracey Ann Kennedy

Founder and Inventor

GENEVIEVE App™



1. Executive summary

GENEVIEVE HEALTH™ Mental Health Practice Support is a proposed connected practice-operations and safety platform. It is designed to prevent staff members, clients, appointments, referrals, communications and urgent matters from being lost across disconnected systems.

Core proposition: the right person sees the right information early enough to act, and no significant task disappears until an accountable outcome is recorded.

The immediate operational problem

- Irene returned from one month of leave without a reliable, consolidated account of what occurred.
- A staff member was away for two weeks without the absence being clearly visible in Irene's return briefing.
- Client, staffing and administrative information appears to become visible only when a problem is already late.
- Responsibilities may be discussed verbally or placed in separate calendars, emails, messages and notes without confirmed acceptance.
- Reception, practitioners and management may each hold only part of the information needed to protect continuity of service.

The proposed solution

- One management command centre showing today's staff, clients, appointments, risks, overdue work and disruptions.
- A mandatory handover and coverage workflow whenever a staff member takes planned or unexpected leave.
- A care-gap register that identifies missed appointments, unreturned contact, unallocated referrals and clients without a next action.
- A reception workspace with safe scripts, callback queues, appointment disruption tools and rapid escalation to an authorised clinician.
- A structured emergency and critical-incident centre covering distress, self-harm concerns, suicide-related events, violence, fire, flood, storms, outages and building closure.
- Role-based access, audit history, privacy controls and explicit limits on automation.

2. Safety and design principles

Principle	Meaning
Human responsibility	The program supports authorised people; it does not replace clinical judgement, diagnosis, treatment, emergency decisions or legal obligations.
No silent handovers	A handover is not complete until the receiving person



	acknowledges responsibility.
No ownerless tasks	Every significant item must have a named primary owner, backup owner, due time and escalation path.
No false closure	A message sent is not an outcome. Closure requires a documented action and result.
Minimum necessary access	Reception, clinicians, managers and external parties see only the information required for their role.
Calm visibility	The interface uses clear green, amber and red signals without overwhelming staff.
Respectful language	Alerts describe facts and actions, not stigmatising labels.
Fail-safe operations	The practice retains paper, telephone and emergency fallback procedures when technology is unavailable.

3. Irene's management command centre

The opening screen should answer the following questions within seconds:

- Who is working, absent, late, remote, on leave or unable to access the practice?
- Who is covering every affected appointment, client, inbox, callback and administrative responsibility?
- Which clients have appointments today, missed an appointment or do not have a next appointment?
- Which communications, referrals, reports, consent forms and follow-ups are overdue?
- Are there red safety alerts, severe-weather disruptions, technology failures or building issues?
- What changed during Irene's absence, what remains unresolved and what requires her personal decision?

Traffic-light status

GREEN	Completed, safe, acknowledged or on track.	Named owner • Due time • Last action • Escalation
AMBER	Approaching deadline, awaiting response or requiring attention.	Named owner • Due time • Last action • Escalation
RED	Urgent, overdue, unassigned, unacknowledged or safety-related.	Named owner • Due time • Last action • Escalation
GREY	Unavailable, paused, not applicable or awaiting external information.	Named owner • Due time • Last action • Escalation



4. Return-from-leave briefing

When Irene or any manager returns, the program should automatically prepare a structured briefing covering:

- Staff absences, current leave, upcoming leave and changed availability.
- Clients transferred between practitioners or temporarily covered.
- Missed appointments, repeated cancellations and clients who did not rebook.
- Reported increases in distress, safety-plan reviews and serious incidents.
- Referrals received, reviewed, declined, accepted or still unallocated.
- Reports, letters, forms and correspondence awaiting completion or signature.
- Unanswered calls, portal messages, emails and shared-inbox items.
- Complaints, privacy concerns, billing problems and external-agency deadlines.
- Policies, procedures or practice arrangements changed during the absence.
- Decisions that cannot be delegated and require Irene's review.

5. Staff support, leave and coverage

Absence scenarios

- Annual leave, personal leave, emergency leave, bereavement leave and planned extended absence.
- Unexpected sickness, no-show, late arrival, hospital admission or sudden resignation.
- Flooded or closed roads, severe storms, public-transport disruption and vehicle breakdown.
- Building access failure, internet outage, telephone outage or inability to access systems remotely.
- A practitioner becoming unwell during a session or a reception worker becoming unavailable during operating hours.

Required automatic workflow

1. Notify the authorised manager and record the expected duration.
2. Identify every affected appointment, client, task, inbox, report and deadline.
3. Separate routine work from clinically sensitive or time-critical work.
4. Propose an authorised covering person and backup.
5. Require the covering person to accept or decline each responsibility.
6. Notify affected clients using an approved, privacy-conscious message.
7. Escalate anything that remains unaccepted.
8. Generate a return handover and confirm completion.

6. Client continuity and care-gap register

The system should bring a client to attention when an approved rule indicates that continuity may have broken, including:

- No next appointment is booked.



- The client cancelled and did not rebook.
- The client did not attend.
- Repeated contact attempts remain unanswered.
- A referral is waiting for review or allocation.
- A report, letter or treatment-plan action is overdue.
- A hospital admission, discharge or change of treating team has been reported.
- The assigned practitioner is on leave and no coverage has been accepted.
- A safety plan requires review.
- A child, vulnerable adult, interpreter user or person with accessibility needs requires a specific arrangement.
- Funding, referral or care-plan documentation is approaching expiry.

Every flag must display the reason, responsible person, permitted next step, due time, last contact and escalation status.

7. Missed appointment and failed-contact workflow

9. Record whether the client or practitioner missed the appointment.
10. Check the client's approved contact method and whether a message may safely be left.
11. Notify the responsible practitioner and create a timed follow-up task.
12. Apply the practice's clinically approved non-attendance protocol.
13. Record each attempted contact and its outcome.
14. Offer appropriate rebooking or transfer options.
15. Escalate according to the approved protocol when a responsible person does not accept the task.
16. Close only after the outcome and next action are documented.

Important: a missed appointment must not automatically be treated as proof of immediate suicide risk, and a lack of reply must never be treated as proof that the person is safe.

8. Distress, self-harm and suicide-safety support

This module must be controlled by Irene's approved clinical policies and authorised practitioners. Its purpose is to place the correct information and escalation steps in front of the responsible person—not to make an autonomous risk decision.

Functions to include

- A clearly visible Immediate Safety Concern control.
- Current location and contact verification prompts.
- Access to the approved safety plan and documented emergency-contact permissions.
- Immediate assignment to the designated practitioner or clinical lead.
- Structured recording of contact attempts, decisions, consultations and outcomes.



- Approved pathways for emergency services or welfare checks where clinically and legally appropriate.
- Mandatory follow-up scheduling and post-incident review.
- Staff debrief, supervision and wellbeing support after a serious event.
- A critical-incident and bereavement process if a death occurs.

The program must never

- Diagnose a client or declare a client safe.
- Determine suicide risk solely from keywords, an artificial-intelligence score or a missed appointment.
- Automatically contact police, family or emergency contacts without an authorised workflow.
- Replace the practitioner's assessment, documentation or professional responsibility.
- Close a red alert merely because an automated message was sent.

9. Reception support centre

Reception requires a restricted operational view that protects clinical confidentiality while making urgent action simple.

- Today's appointment schedule and practitioner attendance status.
- Booking, rebooking, cancellation and clinician-absence workflows.
- Callback and voicemail queues with owner, due time and escalation.
- Identity verification and safe communication preferences.
- Interpreter, accessibility and authorised-representative information.
- Referral, waitlist, intake, report-collection and payment status.
- Approved scripts for distressed, confused, angry or threatening callers.
- One-button escalation to an available authorised clinician.
- System-outage paper procedure and opening/closing checklists.

Reception should never be required to make a clinical risk decision. The system must transfer concerning contact rapidly to an authorised clinician or emergency pathway.

10. Intake, referrals and waitlist safety

New-client intake

- Identity, contact details, referral information and communication preferences.
- Consent, privacy notice, emergency-contact permissions and authorised representatives.
- Accessibility, interpreter and support-person requirements.
- Service suitability review and allocation status.
- Clear explanation that messages are not monitored continuously and the practice is not an emergency service.



Referral control

- Date received, source, stated urgency, documents included and missing.
- Assigned reviewer, suitability decision, allocation and contact attempts.
- Confirmation to the referrer where authorised.
- Escalation of referrals that remain unallocated.

Waitlist boundaries

- Last contact, expected wait communicated and next review date.
- Whether another service is currently supporting the person.
- Alternative service information provided.
- Wording that does not imply active monitoring or treatment before acceptance.

11. Weather, disaster and business-continuity support

The operational emergency centre should cover:

- Storms, flooding, bushfires, extreme heat and unsafe roads.
- Fire, smoke, gas smell, evacuation, lockdown and building inaccessibility.
- Power, internet, telephone, water or air-conditioning failure.
- Public-transport disruption and staff unable to reach work.
- Whole-practice closure or conversion of appropriate appointments to telehealth.
- Violence, threats, duress, medical emergencies and police or ambulance attendance.

Disruption workflow

17. Activate the incident and nominate the incident lead.
18. Confirm staff safety and availability.
19. Identify every affected client, appointment and essential task.
20. Choose approved cancellation, rescheduling, telehealth or relocation actions.
21. Send minimum-necessary communications through approved channels.
22. Review high-priority follow-ups separately.
23. Maintain an incident timeline and decision record.
24. Confirm recovery, reopening and outstanding follow-up.

12. Documents, communications and complaints

Document register

- Session notes and treatment documentation due.
- Safety plans, consent forms and release-of-information forms.
- GP, psychiatrist, NDIS, insurer, school or legal reports.



- Referral responses, discharge summaries and external correspondence.
- Review, signature, delivery and confirmation status.

Communication centre

- Calls, voicemails, SMS, emails, portal messages and letters.
- Identity and consent checks before sensitive information is released.
- Named owner, due time, sensitivity level and outcome.
- Wrong-recipient safeguards and complete audit history.

Complaints and privacy concerns

- Private submission, acknowledgement deadline and responsible investigator.
- Clinical, service, staff-conduct, privacy and accessibility categories.
- Immediate safety screening, conflicts management, response deadline and corrective actions.
- External escalation information where required.

13. Privacy, security and governance requirements

- Individual accounts, strong authentication and no shared passwords.
- Role-based access separating reception, clinical, management and administrative information.
- Automatic logout, encryption, secure backups and tested restoration.
- Full access, change, export and deletion logs.
- Consent records, correction processes, retention rules and staff-exit access removal.
- Data-breach response procedure and authorised notification review.
- Use of fictional or de-identified data until a secure production environment is formally approved.
- Clinical governance, policy ownership, version control, training and supervision records.

14. Artificial-intelligence boundaries

Permitted assistance after approval	Not permitted as an autonomous function
Summarising operational handovers	Diagnosis or treatment decisions
Highlighting overdue tasks	Suicide-risk determination
Drafting routine appointment messages	Declaring a client safe
Organising work queues	Emergency-service decisions
Identifying incomplete form fields	Unreviewed clinical-note generation
Preparing daily operational reports	Sending clinical information to an unapproved external service



15. Recommended controlled first release

The first deployable operational release should concentrate on functions that create immediate visibility without pretending to be a completed clinical information system.

Priority	Module	Initial boundary
1 - Immediate	Irene's management command centre	Operational support only; real clinical data requires approved security, policy and governance.
1 - Immediate	Staff attendance, leave and coverage	Operational support only; real clinical data requires approved security, policy and governance.
1 - Immediate	Return-from-leave briefing	Operational support only; real clinical data requires approved security, policy and governance.
1 - Immediate	Mandatory staff handovers	Operational support only; real clinical data requires approved security, policy and governance.
1 - Immediate	Client care-gap and non-attendance register	Operational support only; real clinical data requires approved security, policy and governance.
1 - Immediate	Reception callback and shared-inbox queue	Operational support only; real clinical data requires approved security, policy and governance.
1 - Immediate	Referral and waitlist tracker	Operational support only; real clinical data requires approved security, policy and governance.
1 - Immediate	Green, amber and red task escalation	Operational support only; real clinical data requires approved security, policy and governance.
2 - Controlled	Immediate Safety Concern control linked to approved procedures	Operational support only; real clinical data requires approved security, policy and governance.
2 - Controlled	Weather, closure and staff/client broadcast controls	Operational support only; real clinical data requires approved security, policy and governance.



2 - Controlled	Critical-incident recording	Operational support only; real clinical data requires approved security, policy and governance.
2 - Controlled	Role-based accounts and audit history	Operational support only; real clinical data requires approved security, policy and governance.
2 - Controlled	Daily opening and closing checklists	Operational support only; real clinical data requires approved security, policy and governance.
2 - Controlled	Backup and export function	Operational support only; real clinical data requires approved security, policy and governance.

16. Scenario coverage register

Area	Scenarios to be covered
Staffing	Planned leave, sickness, no-show, late arrival, emergency leave, sudden resignation, clinician unwell mid-session
Travel and access	Flooded road, storm damage, transport disruption, vehicle breakdown, building inaccessible
Client continuity	Missed appointment, repeated cancellation, no next booking, failed contact, clinician transfer
Referrals	Referral lost in inbox, documents missing, no reviewer, unsuitable service, unallocated referral
Reception	Unanswered voicemail, distressed caller, aggressive caller, privacy-sensitive caller, interpreter need
Clinical safety	Immediate distress, self-harm concern, suicide-related incident, hospital admission/discharge reported
Physical safety	Fire, smoke, medical emergency, violence, threat, duress, evacuation, lockdown
Technology	Power outage, internet failure, telephone failure, system outage, data restore
Privacy	Wrong recipient, unauthorised access, lost device,



	data breach, consent uncertainty
Governance	Overdue note, unsigned report, expired credential, missed supervision, outdated procedure
Critical incidents	Serious injury, death, police/ambulance attendance, complaint, staff debrief and corrective action

17. Decisions required from Irene

Before any real client or staff information is entered, Irene should approve or nominate:

- The practice's authorised clinical lead and emergency escalation hierarchy.
- Existing non-attendance, distress, self-harm, suicide-safety and critical-incident procedures.
- Which roles may see clinical, contact, scheduling, billing and incident information.
- Approved communication wording and when voicemail, SMS or email may be used.
- Staff leave, handover and coverage responsibilities.
- Business-continuity procedures for weather, closure and technology outages.
- The minimum information required for a safe first trial.
- Who will conduct legal, privacy, cybersecurity and clinical-governance review.
- Whether the initial demonstration will use fictional, de-identified or real information.

18. Proposed next step

Hold a structured discovery meeting with Irene and one representative from reception and clinical staff. Walk through the practice's current process from staff absence to client contact, identify the points where information is lost, map the approved escalation rules, and configure the first controlled release around those real workflows.

GENEVIEVE App™ operating rule

**DETECTED → ASSIGNED → ACCEPTED → ACTIONED → OUTCOME RECORDED → REVIEWED
→ CLOSED**

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